

1. Administrative & Study Identification

Full Study Title: A Multicenter, Randomized, Double-blind, Placebo-controlled, Phase 3 Study to Evaluate the Efficacy, Safety, and Tolerability of BMS-986278 in Participants With Idiopathic Pulmonary Fibrosis **Short Title/Acronym:** ALOFT-IPF **Protocol Number:** IM027068 **NCT Number:** NCT06003426 **Sponsor Name:** Bristol-Myers Squibb **Investigational Product:** BMS-986278 (Admilparant), an oral lysophosphatidic acid receptor 1 (LPA1) antagonist **Phase of Development:** Phase 3 **Medical Monitor:** Bristol-Myers Squibb Clinical Trials Operations (clinical.trials@bms.com)

2. Study Synopsis & Rationale

2.1 Study Objective Primary Objective: To evaluate the efficacy, safety, and tolerability of the LPA1 antagonist BMS-986278 (Admilparant) compared to placebo in participants with Idiopathic Pulmonary Fibrosis (IPF). **Secondary Objectives:** To evaluate time to disease progression, long-term survival, and health-related quality of life over 52 weeks of treatment.

2.2 Background & Rationale There remains a large unmet medical need for a safe, well-tolerated, and effective therapy for idiopathic pulmonary fibrosis (IPF) that improves pulmonary function, delays disease progression, and reduces mortality. Increased lysophosphatidic acid (LPA) levels and activation of the LPA1 receptor are directly involved in the pathogenesis of pulmonary fibrosis. This trial investigates whether BMS-986278 can safely inhibit this pathway and slow forced vital capacity (FVC) decline in IPF patients.

3. Study Design & Methodology

3.1 Design Framework Study Type: Interventional **Control Type:** Placebo-controlled **Blinding:** Double-blind (with a single-blind Cohort 1 for initial safety) **Randomization:** 1:1:1 (Admilparant 60 mg vs. Admilparant 120 mg vs. Placebo)

3.2 Planned Enrollment & Duration Target \$N\$: Approximately 1,185 participants (Cohort 1: ~60; Cohort 2: ~1,125+) **Number of Sites:** >400 sites across >30 countries **Estimated Study Start:** 14-Sep-2023 **Estimated Primary Completion:** 06-Oct-2026

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria 1. Subjects with IPF aged ≥ 40 years at the time of signing the informed consent. 2. Diagnosis of IPF within 7 years prior to screening that is supported by centrally read chest high-resolution computed tomography (HRCT) obtained at screening and verification of usual interstitial pneumonia. 3. If on pirfenidone or nintedanib, participants must have been on a stable dose for at least 90 days prior to screening. If not currently on pirfenidone or nintedanib, participants must not have received either of these medications within 28 days prior to screening. 4. Women of childbearing potential must have a highly effective form of contraception and must provide a negative pregnancy test. Men who are sexually active with women of childbearing potential must agree to use barrier contraception.

4.2 Exclusion Criteria 1. History of stroke or transient ischemic attack within 3 months prior to screening. 2. Participants who exhibit symptoms of heart failure at rest. 3. Participants who have a current malignancy or a previous malignancy with less than 2 years free of recurrence.

5. Intervention & Treatment Plan

5.1 Investigational Regimen Dosage/Frequency: BMS-986278 60 mg or 120 mg twice daily (BID), including a 7 to 16 days dose titration period starting at Day 1. **Route of Administration:** Oral **Duration of Treatment:** Minimum of 52 weeks (up to approximately 4 years for extended follow-up).

5.2 Concomitant & Prohibited Therapy Permitted: Stable background antifibrotic therapy for IPF (nintedanib or pirfenidone).

Prohibited: Unstable use of antifibrotics within 28 days prior to screening; treatments that have a known high risk for interaction not explicitly cleared by the protocol.

6. Study Timeline & Visit Schedule

Visit ID	Window (Days)	Key Activities/Procedures
Screening	Day -28 to Day -1	Informed Consent, Medical History, HRCT Scan, Spirometry (FVC)
Baseline (Day 1)	Day 1	Randomization, First Dose, Titration Initiation
Interim Visits	Q6W up to Wk 52, then Q12W to EOT	Efficacy Assessments (Spirometry), Safety Labs, Pharmacokinetics, 6MWT
End of Treatment	Week 52 / EOT	Final 52-week FVC Assessment, L-PF Questionnaire, Quality of Life scales
Follow-up	EOT + 28 Days	Long-Term Safety Follow-up, Ongoing Evaluation for Mortality

7. Outcome Measures (Endpoints)

7.1 Primary Endpoint Definition: Cohort 1: Number of spontaneous syncopal episodes from Day 7 to Day 29 post-treatment. Cohort 2: Absolute change in Forced Vital Capacity (FVC) from baseline to Week 52. **Measurement Method:** Central spirometry reviews and safety reporting.

7.2 Secondary & Exploratory Endpoints - Time to disease progression - Change in Living with Pulmonary Fibrosis (L-PF) questionnaire scores - Long-term safety and overall survival up to ~3 years - Pharmacokinetics and 6-Minute Walk Test (6MWT) evaluations over time

8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Standard collection including blood pressure monitoring, safety labs, and evaluating for spontaneous syncopal episodes. **Serious Adverse Events (SAEs):** Safety reviews conducted regularly to evaluate treatment-emergent SAEs and mortality. **Data Monitoring Committee (DMC):** An unblinded independent review committee assessed Cohort 1 safety before opening the Phase 3 global Cohort 2.

9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: Intent-to-Treat (ITT) population for primary efficacy analysis (absolute change in FVC); Safety Set for all AE/SAE evaluations. **Sample Size Justification:** \$N \approx 1,125\$ for Cohort 2 powered to detect statistically significant treatment differences in FVC decline between BMS-986278 (60 mg or 120 mg) vs. placebo at 52 weeks. **Handling of Missing Data:** Standard handling utilizing mixed-effect models for repeated measures (MMRM) to address dropouts and missing FVC data across the 52-week timeframe.

10. Quality Assurance & Regulatory Compliance

GCP Compliance: This study will be conducted in accordance with Good Clinical Practice (GCP) and the Declaration of Helsinki. **Monitoring Plan:** Comprehensive global site monitoring, unblinded safety reviews (Cohort 1 to Cohort 2 transition), and centralized spirometry and HRCT imaging reading. **Audit Trail:** Source data verification and eCRF queries per standard global Phase 3 protocols.

11. Study Identifiers & Governance

Primary ID: IM027068 **Registry Number:** NCT06003426 **Sponsor/Lead Organization:** Bristol-Myers Squibb **Study Title:** ALOFT-IPF
Official Regulatory Title: A Multicenter, Randomized, Double-blind, Placebo-controlled, Phase 3 Study to Evaluate the Efficacy, Safety, and Tolerability of BMS-986278 in Participants With Idiopathic Pulmonary Fibrosis

12. Clinical Framework (Idiopathic Pulmonary Fibrosis Specific)

Primary Condition: Idiopathic Pulmonary Fibrosis (IPF) **Diagnosis Threshold:** Clinically confirmed diagnosis within 7 years, supported by HRCT verification of usual interstitial pneumonia (UIP). **Medical Methodology:** Lysophosphatidic acid receptor 1 (LPA1) antagonism. **Optimization Target:** Slowing the decline of percent predicted FVC (ppFVC) / absolute FVC over 52 weeks.

13. Study Procedures & Methodology

Management Pathway: Protocol-specified stepwise titration (7 to 16 days) to target maintenance dose (60 mg or 120 mg BID). **Education Protocol (HCP):** Standardization training for spirometry testing and HRCT imaging acquisition. **Education Protocol (Patient):** Symptom tracking, medication adherence, and reporting of syncopal episodes. **Quality Audit Mechanism:** Independent central imaging reading and unblinded safety evaluations.

14. Observation & Follow-Up Schedule

Total Duration: ≥ 52 Weeks (up to 4 years inclusive of long-term follow-up) **Screening Phase:** Day -28 to Day -1 for informed consent, medical history, HRCT scan, and spirometry (FVC). **Baseline:** Day 1 for randomization, first dose, and titration initiation. **Onsite Visit Cadence:** Interim visits every 6 weeks (Q6W) up to Week 52, then every 12 weeks (Q12W) to EOT. Activities include efficacy assessments (spirometry), safety labs, pharmacokinetics, and the 6-minute walk test (6MWT). **End of Treatment (EOT):** Week 52 / EOT includes the final 52-week FVC assessment, L-PF questionnaire, and quality of life scales. **Follow-Up Window:** EOT + 28 Days for long-term safety follow-up and ongoing evaluation for mortality. **Remote Monitoring Frequency:** Regular safety touchpoints as per site clinical protocols. **Checklist Review Interval:** Continuous monitoring for AEs and concomitant medications.

15. Sign-off & Approval

Role	Name	Signature	Date		:---	:---	:---	:---
Principal Investigator	[Name]	_____	[DD-MMM-YYYY]					
Clinical Operations Lead	[Name]	_____	[DD-MMM-YYYY]					Lead
Statistician	[Name]	_____	[DD-MMM-YYYY]					